



Introduction :

Maternal healthcare utilization is a major determinant of maternal mortality. Bangladesh has made tremendous improvements in essential healthcare indices like maternal, neonatal, and under-5 mortality in recent years. Although the yearly average decreasing rate in the maternal mortality ratio (MMR) was relatively high between 2000 and 2015, it was still insufficient to meet the MMR target by the deadline for the MDGs. Bangladesh is experiencing development in many aspects, yet rural regions are lagging behind urban areas in many ways. Healthcare infrastructure is also different in urban and rural areas. And there is substantial inequality exists at various socioeconomic factors that contribute to the significantly large share of inequality in utilization of maternal health care in Bangladesh.

Objective:

- This study aimed to assess the socioeconomic inequalities in the utilization of maternal health care in urban-rural areas.

Methods:

- In this study we used Secondary data from Bangladesh Demographic and Health Survey (BDHS) conducted in 2017-18.
- For the most recent live birth, a total of 5052 women aged between 15–49 years old, who had given birth three years before the survey are included in the study.
- The study's findings are related to the utilization of Antenatal care (ANC), Facility-Based Delivery (FBD), and Skilled Birth Attendants (SBA)—three crucial areas of maternal healthcare—during the most recent pregnancy and birth.
- Urban-rural inequalities in the utilization of maternal health care services have been calculated.
- We performed Chi-square test to examined the association between the three factors of maternal care services and some socioeconomic variables.

Fig-1: USES OF MATERNAL CARE SERVICES IN URBAN-RURAL AREAS

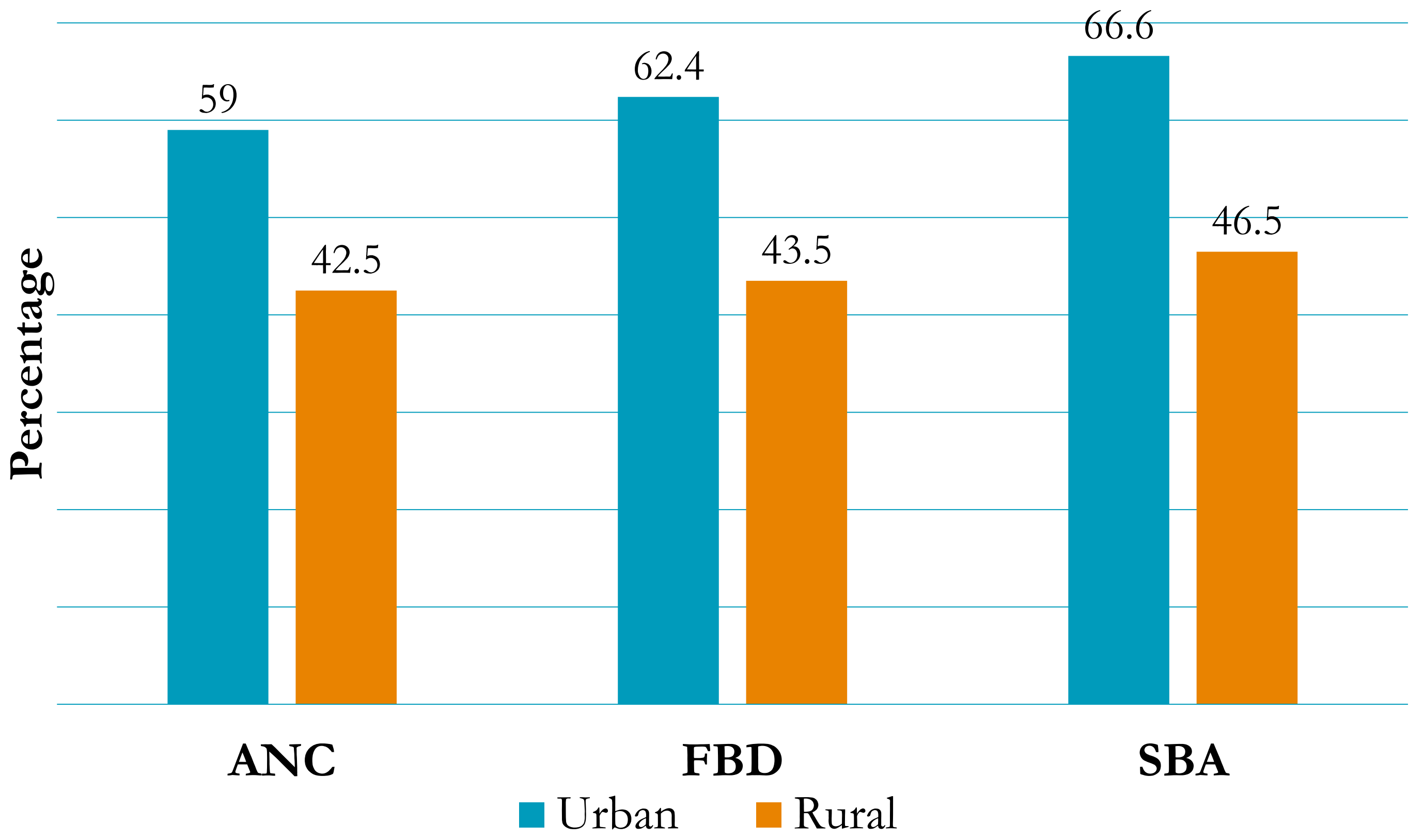


Fig-2: PERCENTAGE OF WOMEN HAVING ANC DIVISIONALLY

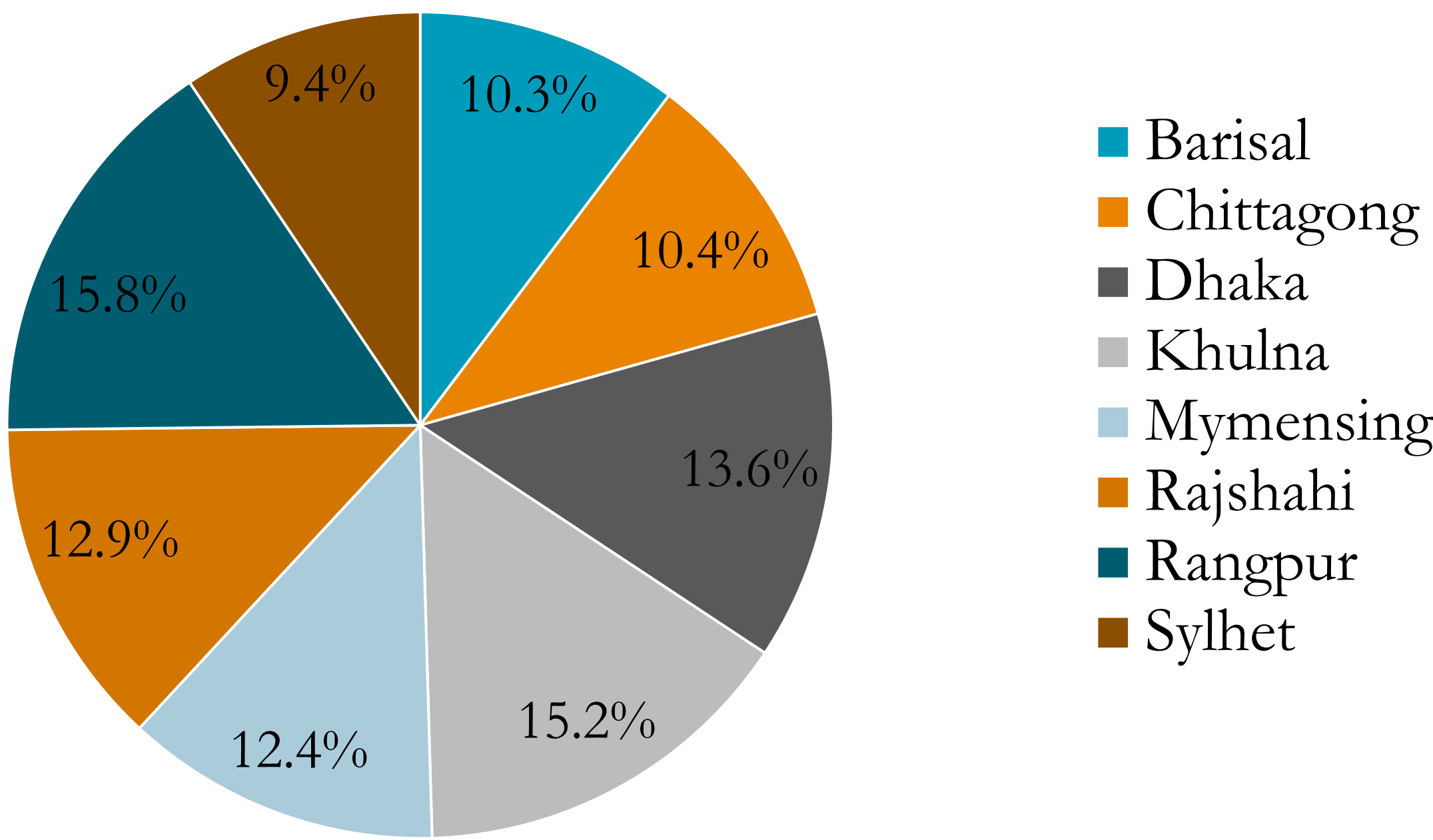


Table 1: DESCRIPTIVE ANALYSIS AND THE CHI-SQUARE TEST

Covariate	ANC		P-Value	FBD		P-value	SBA		P-value
	Urban	Rural		Urban	Rural		Urban	Rural	
Mother's Age									
15-19	154(55%)	262(44%)	0.012	153(54%)	281(47%)	0.00	168(60%)	301(51%)	0.00
20-24	349(60)	509(42)		353(61)	565(47)		385(66)	599(50)	
25-34	450(60)	560(42)		492(65)	521(39)		513(68)	555(52)	
Partner's education level									
Primary	237(47)	400(34)	0.00	227(45)	405(34)	0.00	247(50)	436(37)	0.00
Secondary	335(59)	514(47)		362(64)	532(49)		384(68)	570(52)	
Higher	374(78)	320(65)		410(86)	346(71)		433(90)	360(74)	
Mother's Education									
No education	25(25)	38(17)	0.00	34(34)	44(20)	0.00	35(35)	50(23)	0.00
Primary	168(40)	308(31)		163(38)	276(28)		175(41.7)	300(30)	
Secondary	480(62)	756(46)		487(63)	782(47)		529(69.1)	833(50)	
Higher	345(78)	294(63)		392(88)	329(70)		409(92.7)	345(74)	
Household Wealth index									
Poorest	58(31)	275(30)	0.00	37(19)	241(26)	0.00	41(22)	257(28)	0.00
Poorer	65(44)	322(36)		71(48)	311(35)		76(52)	338(38)	
Middle	117(49)	316(47)		120(50)	324(48)		133(56)	351(52)	
Richer	235(52)	297(54)		264(59)	325(59)		282(63)	345(63)	
Richest	543(76)	186(59)		584(82)	230(73)		616(86)	237(75)	
Household Head Occupation									
Framer	267(56)	564(40)	0.00	285(60)	539(38)	0.00	305(64.2)	583(42)	0.00
Worker	519(55)	683(41)		537(57)	713(43)		574(61.0)	759(46)	
Job & Business	213(79)	120(62)		233(87)	137(70)		246(62.1)	142(73)	
Mother's Working after marriage									
No	235(62)	557(46)	0.00	343(65)	581(48)	0.00	368(70)	617(51)	0.00
Yes	238(77)	235(65)		258(84)	243(67)		271(88)	260(72)	

Results:

1. We see from the graph that maternal health services greatly vary in urban and rural areas. In urban areas, 66.6% of women have SBA while in rural areas have 46.5% during their delivery.
2. In different divisions, the percentage of receiving ANC, FBD, and SBA differ greatly. In our pie chart, we can see the percentage of receiving ANC in the Sylhet divisions(9.4%) is lower compared to the Khulna divisions(15.2%).
3. In the household wealth index, the proportion of women from the richest and poorest quintiles that used ANC is almost three times higher in urban areas. For the purpose of wealth index ANC, FBD, and SBA highly differ between the richest and poorest which an approximate percentage goes to 25% to 75%.
4. All socio-demographic variables have a significant effect on maternal health care services; of them, maternal education, Partners education is the most important factor. Observing the table, ANC, FBD, and SBA are depended on the level of the mother's & partners education. In urban areas, Illiterate women received 25% ANC which is 3 times lower than higher-educated women (78%). Both FBD and SBA are proportionally differ in urban and rural areas along through the categories of education level.
5. All of the above variables have been found statistically significant and had a great influence on the use of maternal healthcare services.

Conclusions and Recommendation :

The findings of this study provide insights for planning and implementing appropriate maternal health service programs to improve the health and well-being of both mother and child. Thus, priority focus should be given to implementing and evaluating interventions that benefit women who are poorer, less educated and live in rural areas.